

Cardiology Summary · Synthesized by Claude — reference reports below are clickable / viewable

Compiled from inpatient records — Sir H. N. Reliance Foundation Hospital, Mumbai · Generated 29 April 2026

Patient: Saroj Agarwal · **Sex/Age:** Female / 63 y · **DOB:** 27 Aug 1962 · **Patient ID:** 0010663186 · **Cardiologist on file:** Dr. Talha Meeran (FACC) · **Oncologist:** Dr. Darshit Shah

Reason for review: Severe Group III pulmonary hypertension with worsening RV function (FAC 20 %) post-saddle submassive pulmonary embolism (24 Mar 2026); on Apixaban + Amiodarone + Mexiletine + Tadalafil. History of polymorphic PVCs and runs of NSVT during admission, now controlled. Holter (14–17 Mar 2026) showed multifocal ventricular ectopy with 60 episodes of bigeminy, no NSVT. NT-proBNP rose from 1761 → 4089 pg/mL across the admission. Background of metastatic carcinoma uterus on chemoimmunotherapy.

1 · Background

- **Primary diagnosis:** Carcinoma of uterus (C54), operated 3 years prior; current admission for metastatic recurrence.
- **Index hospitalization:** 20 Feb → 8 Apr 2026 (47-day inpatient). Admitted with breathlessness, cough, bilateral pedal oedema, left pleural effusion (ICD *in situ* from 12 Feb 2026, removed 24 Feb 2026).
- **Active oncology plan:** Paclitaxel + Carboplatin + Dostarlimab; last cycle C3 D8 on 13 Apr 2026.

2 · Cardiac issues

A. Severe Group III pulmonary hypertension with RV strain/dysfunction

Date	Echo finding
20 Feb 2026	Severe PAH, RVSP 83 mmHg , Grade I+ TR (eccentric jet); MPA 27 mm; mildly dilated RV (39×40×61 mm) with fair RV systolic function (TAPSE 16 mm); LV normal size with mild systolic septal flattening (RV pressure overload); LVEF 55 % ; Grade I LV diastolic dysfunction (E/E' 7); structurally normal valves; normal IVC (10 mm) with 50 % collapse; no pericardial effusion.
26 Feb 2026 (pre-op for vocal-cord lateralization)	Severe PAH, RVSP 72 mmHg , mild TR; MPA 25 mm; mildly dilated RV (37×40×53 mm) with normal RV function (TAPSE 21 mm); LVEF 60 %; Grade I LV diastolic dysfunction (E/E' 5); right-sided pleural effusion. (<i>Reported by Dr. Talha Meeran.</i>)
1 Mar 2026 (POD-3 screening per discharge note)	LVEF 60 %, mildly dilated RV with normal function, mild TR, PASP ~46 mmHg (improved).
24 Mar 2026 (post-PE)	Strong suspicion of PE — large saddle thrombus at the bifurcation of RPA & LPA . Severe PAH, moderate-severe TR, RVSP 91 mmHg , PAT ~91 ms; dilated RA (40 mm) and RV (50×45×77 mm) with RV systolic dysfunction — FAC 20 % ; LVEF 60 % with septal flattening and good contractility; mild LV diastolic dysfunction (E/E' 6); IVC 16 mm with >50 % collapse. (<i>Reported by Dr. Maulik Parekh, Interventional Cardiologist.</i>)

B. Saddle submassive pulmonary embolism (24 Mar 2026)

- **CTPA 24 Mar 2026:** Saddle thrombus extending from main pulmonary trunk into both R and L main PAs; complete occlusion of proximal left upper-lobe segmental branch; partial filling defects in right distal PA, all basal segments of RLL, and RML/RUL sub-segmental branches; pulmonary trunk 35 mm; bilateral moderate pleural effusions; RA & RV mildly dilated; **straight interventricular septum (RV strain)**.
- **Course:** Clexane initiated 24 Mar; re-intubation, ICU, vasopressor support; ET-tube bleeding + Hb drop 27–29 Mar required **anticoagulation hold** with transfusions; cautious restart 31 Mar after platelet recovery. Note: discharge summary mentions "post pulmonary thrombectomy status" on 4 Apr — interventional record not seen in this dataset; please verify with treating team if a thrombectomy was performed.
- **Discharge anticoagulation:** Clexane 0.6 mL SC BD × 14 days → then **Apixaban 5 mg PO BD**.

C. Ventricular arrhythmias

- 6 Mar 2026: Cardiology review noted **frequent PVCs and NSVT** → Cordarone (Amiodarone) + Concor (Bisoprolol) initiated.
- 8 Mar 2026: Episodes of **polymorphic PVCs and VTach with palpitations**; managed with **Xylocard (Lidocaine) infusion** and anti-arrhythmics. Coronary angiography considered then deferred after cardiology discussion.
- 12–13 Mar 2026: Intermittent ventricular ectopy / bigeminy on telemetry; aggressive electrolyte correction (target K⁺ >4 mmol/L, Mg²⁺ >2 mg/dL).
- **SmartCardia Holter, 14–17 Mar 2026 (3 days, 2 d 23 h analysed):** Min HR 52 bpm, max 106 bpm, avg 75 bpm; QRS 325 590; SVE 5 (all isolated); **VE 1811 (1809 isolated, 1 couplet, 0 runs); 60 episodes of ventricular bigeminy; 0 trigeminy**; no NSVT, no AF, no AV block, no pauses (longest RR 1.31 s). **Final interpretation:** "Underlying normal sinus rhythm with occasional multifocal ventricular ectopies along with brief runs of ventricular bigeminy. No NSVT or VTACH." Ordering physician: Dr. Talha Meeran.
- 20 Mar 2026: MET call for chest pain — managed symptomatically; ECG advised; cardiac meds continued.
- By discharge: **no ventricular arrhythmias for >48 h** on Cordarone + Mexiletine + Tadalafil + Aldactone.

- **Note on 14 Mar ECG:** the formal ECG report on file (Performed 14/03/2026) contains only "na" in its findings field, signed by the technician (no cardiologist interpretation captured) — surrounding telemetry/Holter data covers this gap.

D. Cardiac biomarkers

- **NT-proBNP:** 20 Feb 2026 = **1761 pg/mL** (ref 0–125; >14× ULN); 24 Mar 2026 = **4089 pg/mL** (peri-PE; further rise consistent with RV strain).
- **Troponin I-hs:** 8 Mar 2026 = **3.2 pg/mL** (normal); 25 Mar 2026 = **11.90 pg/mL** (still < ref ≤15.6 pg/mL — borderline, used to assess thrombolysis need; thrombolysis not pursued).

3 • Discharge cardiac regimen (Dr. Talha Meeran, 8 Apr 2026)

Medication	Dose / schedule
Clexane (Enoxaparin) 0.6 mL SC BD	For 14 days, then switch
Apixaban 5 mg PO BD	From day 15 onwards (PE on active malignancy)
Cordarone (Amiodarone) 200 mg PO BD	Anti-arrhythmic — continue
Mexohar (Mexiletine) 150 mg PO TDS	Anti-arrhythmic — continue
Tadalafil 20 mg PO HS	For pulmonary hypertension — continue
Aldactone (Spironolactone) 50 mg PO OD (0-1-0, afternoon)	Diuresis / RV unloading — continue

4 • Most recent labs (13 Apr 2026 daycare) & key trajectory

- **13 Apr CBC:** Hb **8.3 g/dL** (low; ref 12.0–15.0); **RBC 2.82 ×10¹²/L**; HCT 25.7 %; RDW 19.7 % (high); MPV 11.0 fl (high); platelets 199 ×10⁹/L; TLC 4.02 ×10⁹/L (normal).
- **Late-March CBC nadirs** (peri-PE, on/off anticoagulation): **Hb 7.3 (29 Mar)**; **Platelet 51 ×10⁹/L (29 Mar)**; **TLC 1.43 ×10⁹/L (31 Mar)**. All recovered by 13 Apr.
- **13 Apr renal:** BUN 13.36; Urea 28.60; **creatinine 0.63 mg/dL**; **eGFR >90 mL/min/1.73 m²**.
- **13 Apr electrolytes:** Na 137.20, K 3.57, Cl 99.30, HCO₃ 30.00 (high) mmol/L; hypocalcaemia (Ca 7.99 mg/dL).
- **4 Apr 2026 electrolyte excursion** (lab 04/04 04:41, on diuretics): **Na 151.20 mmol/L (high)**, **K 3.40 mmol/L (low)**, **Cl 111.10 mmol/L (high)** — relevant to arrhythmia threshold and volatility seen across admission.
- **Persistent hypoalbuminemia / hypoproteinemia:** Albumin 2.91 (27 Mar) → 2.73 (13 Apr); Total Protein 4.74 → 4.63 g/dL — consider for drug binding (Apixaban, Amiodarone).

Specific questions for Cardiology:

- Targeted PAH escalation — RVSP 91 mmHg with FAC 20 % post-PE: should an ERA or prostacyclin pathway agent be added to Tadalafil?
- Verify if pulmonary thrombectomy was performed during the 25 Mar – 4 Apr ICU stay (referenced once in discharge summary, not corroborated by procedure report) — material for procedural follow-up.
- Anticoagulation duration — provoked PE on active malignancy: **indefinite Apixaban?**
- Repeat Holter and echo to document arrhythmia burden and RV recovery on optimised regimen.
- Long-term plan for Mexiletine (toxicity surveillance) and Amiodarone (TFTs, LFTs, baseline pulmonary function — noting active lung disease).
- Coronary angiography was deferred 8 Mar — re-evaluate need given the polymorphic VT episode.

Source documents:

- **DISCHARGE** [report_pdf/2026-04-08_Summary-Discharge_Discharge-Summary.pdf](#)
- **ECHO** [report_pdf/2026-03-24_ECHO_Echocardiography.pdf](#) · [report_pdf/2026-02-26_ECHO_Echocardiography-Portable.pdf](#) · [report_pdf/2026-02-20_ECHO_Echocardiography-Portable.pdf](#)
- **CTPA** [report_pdf/2026-03-24_CT_CT-Angiography-Pulmonary.pdf](#) — saddle PE, RV strain
- **HOLTER** [report_pdf/2026-03-14_Holter_Holter-ECG-Report.pdf](#)
- **ECG** [report_pdf/2026-03-14_ECG_ECG-Report.pdf](#) (no findings entered — see note above)
- **NT-proBNP** [report_pdf/2026-03-24_Lab-Serol_NT-pro-B-type-Natriuretic-Peptide-NT-pro-BNP.pdf](#) · [report_pdf/2026-02-20_Lab-Serol_NT-pro-B-type-Natriuretic-Peptide-NT-pro-BNP.pdf](#)
- **TROPONIN** [report_pdf/2026-03-25_Lab-Biochem_Troponin-I-hs.md](#) · [report_pdf/2026-03-08_Lab-Biochem_Troponin-I-hs-Less-than.pdf](#)
- **DAYCARE** [report_pdf/2026-04-13_Summary-Daycare_Daycare-Discharge-Summary-Chemotherapy.pdf](#)
- **RECENT LABS** [report_pdf/2026-04-13_Lab-Haem_Complete-Blood-Count.pdf](#) · [report_pdf/2026-04-13_Lab-Biochem_Renal-Profile.pdf](#) · [report_pdf/2026-04-13_Lab-Biochem_Liver-Profile.pdf](#)

Master index: [MASTER_INDEX.md](#). All 188 reports as one file: [ALL_MD_REPORTS.md](#).

This handoff note was assembled from the patient's electronic record (188 reports, 20 Feb – 13 Apr 2026). All facts are sourced from the original PDFs cited above. Please cross-verify against the source PDFs before any clinical decision.